

Section 1904.3 provides that OSHA will accept injury and illness records in place of the employer's Part 1904 records under two circumstances: (1) if OSHA has entered into a memorandum of understanding (MOU) with that agency that specifically accepts the other agency's records, then the employer may use them in place of the OSHA records; or (2) if the other agency's records include the same information required by Part 1904, because OSHA would consider them an acceptable substitute.

D. Forms.

Section 1904.29 directs employers who are required to keep OSHA injury and illness records to use three forms for recordable injuries and illnesses: Form 300 (also called the OSHA 300 Log), Log of Work-Related Injuries and Illnesses, Form 300A, Summary of Work-Related Injuries and Illnesses, and Form 301, Injury and Illness Incident Report, or equivalent forms. Employers are required to keep separate OSHA 300 Logs for each establishment that is expected to be in operation for one year or longer. Under section 1904.31, the OSHA 300 Log must include injuries and illnesses to employees on the employer's payroll as well as injuries and illnesses of other workers the employer supervises on a day-to-day basis, such as temporary workers or contractor workers who are subject to daily supervision by the employer.

Note: An equivalent form is one that has the same information, is as readable and understandable, and is completed using the same instructions as the OSHA form it replaces. Some employers use an insurance form instead of the OSHA Form 301 or supplement an insurance form by adding any additional information required by OSHA. (See section 1904.29(b)(4) and FAQ [29-4](#)).

E. Determining the Recordability of Fatalities, Injuries, or Illnesses.

Section 1904.4(a) mandates that each employer who is required by OSHA to keep records must record each fatality, injury or illness that meets the following: (1) it is work-related; (2) it is a new case and not a continuation of an old case; and (3) it meets one or more of the general recording criteria in section 1904.7 or the additional criteria for specific cases found in sections 1904.8 through 1904.11. Fatalities, injuries, and illnesses that meet these criteria must be recorded on the employer's OSHA Forms 300, 300A, and 301, or equivalent forms.

1. **Work Relationship.** Section 1904.5(a) states that "[the employer] must consider an injury or illness to be work-related if an event or exposure in the work environment either caused or contributed to the resulting condition. Work-relatedness is presumed for injuries and illnesses resulting from events or exposures occurring in the work environment, unless an exception in [section] 1904.5(b)(2) specifically applies." Under this provision, a case is presumed work-related if, and only if, an event or

exposure in the work environment is a discernable cause of the injury or illness, or of a significant aggravation to a pre-existing condition. The work event or exposure need only be one of the discernable causes; it need not be the sole or predominant cause.

Under section 1904.5(b)(3), if it is not obvious whether the precipitating event or exposure occurred in the work environment or elsewhere, the employer "must evaluate the employee's work duties and environment to decide whether or not one or more events or exposures in the work environment caused or contributed to the resulting condition or significantly aggravated a pre-existing condition." This means that the employer must determine whether it is more likely than not that work events or exposures were a cause of the injury or illness, or of a significant aggravation to a pre-existing condition. (As also discussed under section XIV. of this instruction, Citations and Penalties for Violation of Part 1904 Requirements, if the employer decides the case is not work-related, but OSHA disagrees and issues a citation for failure to record, then OSHA will have the burden of proving that the fatality, injury, or illness case was work-related.)

Exceptions. Situations in which the employee's injury or illness meets one or more of the listed exceptions in section 1904.5(b)(2) are not considered work-related and are therefore not recordable. Specifically, OSHA recording requirements do not apply if:

- a. The employee is present as a member of the general public, rather than as an employee, at time of injury or illness;
- b. Symptoms arise in a work environment that are solely due to non-work-related event or exposure; (Regardless of where signs or symptoms surface, a case is work-related only if a work event or exposure is a discernable cause of the injury or illness or of a significant aggravation to a pre-existing condition. See OSHA's January 13, 2004 LOI regarding section 1904.5, [Determining work-relatedness when the work event or exposure is only one of the discernable causes; not the sole or predominant cause.](#))
- c. The injury or illness results solely from voluntary participation in wellness program, medical, fitness or recreational activity;
- d. The injury or illness results solely from eating, drinking, or preparing food or drink for personal consumption;
- e. The injury or illness results solely from personal tasks outside assigned working hours;
- f. The injury or illness results solely from personal grooming, self-medication for non-work-related condition, or intentionally self-inflicted;

- g. The injury or illness is caused by a motor vehicle accident in the parking lot or company access road during commute;
- h. The illness is the common cold or flu; or
- i. The illness is a mental illness, unless the employee voluntarily provides the employer with an opinion from a physician or licensed health care professional (PLHCP) with appropriate training and experience (i.e. a psychiatrist, psychologist, psychiatric nurse practitioner, etc.) stating that the employee has a mental illness that is work-related, and that the case meets one or more of the general recordkeeping criteria in section 1904.7.

Note: Exposures to substances or communicable disease that do not result in signs or symptoms of an injury or illness are not recordable under Part 1904 even if they involve medical treatment beyond first aid, days away from work, restricted work activity or job transfer. (For example, see OSHA's September 14, 2017 LOI, [Determining if the employees experienced an injury or illness due to an exposure.](#))

- 2. **New Case.** Only new cases are recordable. Section 1904.6 provides that work-related injuries and illnesses are considered to be new cases when: (1) the employee has not previously experienced a recorded injury or illness of the same type that affected the same part of the body; or (2) the employee previously experienced a recorded injury or illness of the same type that affected the same part of the body but had recovered completely (all signs and symptoms of the previous injury or illness had disappeared) from the previous injury or illness and an event or exposure in the work environment caused the signs or symptoms to reappear.

As with employer determinations of work-relatedness under section 1904.5, employers may (but are not required to) seek the advice of a physician or other licensed healthcare professional (PLHCP) to determine whether an injury or illness is a new case or recurrence. (See section 1904.6(b)(3).)

- 3. **General Recording Criteria.** Employers must record work-related injuries and illnesses that meet one or more of the general recording criteria in section 1904.7 or the additional criteria for specific cases found in sections 1904.8 through 1904.11. A work-related injury or illness meets the general recording criteria when it results in one or more of the following:
 - a. Death
 - b. Days Away from Work
 - c. Restricted Work
 - d. Transfer to Another Job

- e. Medical Treatment Beyond First Aid
- f. Loss of Consciousness
- g. Diagnosis of a Significant Injury or Illness

These recording criteria are described in greater detail below.

4. **Multiple Applicable Recording Criteria.** Although most cases are recorded because they meet one of these criteria, some cases may meet more than one criterion. The outcomes listed on the OSHA Form 300 include, death, days away, restricted work/job transfer, and "other recordable." Employers must classify each case on the OSHA 300 Log in accordance with the most serious outcome of the injury or illness.

For example, an injured worker may initially be sent home to recuperate (making the case recordable as a "days away" case) and then subsequently return to work on a restricted ("light duty") basis (meeting a second criterion, that for restricted work). For cases that result in both days away from work and days of restricted work activity/job transfer, the case must be classified as days away from work (Column H on the Form 300 Log). For cases resulting in either days away from work or restricted work/job transfer, the employer must track the number of calendar days involved and enter the total on the OSHA 300 Log. The employer may stop counting when the total number of days away from work or restricted work activity/job transfer, or combination of both, reaches 180 calendar days.

5. **Counting Days.** The OSHA 300 Log requires the employer to note the number of days that the employee is unable to work as a result of the injury or illness, or the number of days that a workers is under restricted work, or transferred to another job. Section 1904.7(b)(3)(i) states that the employer is not to count the day of injury or illness, but is to begin counting days away on the following day. Section 1904.7(b)(3)(vii) provides a cap of 180 days. Employers must count the number of calendar days the employee was unable to work as a result of the injury or illness regardless of whether the employee was scheduled to work on those days, including weekend days, holidays or other days off. (See section 1904.7(b)(3)(iv).) Under section 1904.7(b)(3)(vii), the employer may stop counting days away from work if an employee who is away from work because of an injury or illness leaves the company for some reason unrelated to the injury or illness, such as retirement or a plant closing. Cases involving restricted work or transfers are counted in the same way as cases involving days away from work, except that if an employee is permanently assigned to a new or modified job, the

employer may stop the day count (although at least one day of restricted work or job transfer must be counted) (See 1904.7(b)(4)(xi).)

6. **Most Authoritative Medical Opinion.** In cases where two or more physicians or other licensed health care professionals (PLHCP) make conflicting or differing recommendations related to recordability of an employee injury or illness, the employer must make a decision as to which recommendation is the most authoritative (best documented, best reasoned, or most persuasive), and record the case based on that recommendation. This concept applies to cases that involve determinations as to whether an injury or illness is a new case, or whether an injury or illness results in days away from work or restricted work activity.

It is imperative that the PLHCPs involved in the examination of the employee see the exact same condition. If the employee's condition either improves or worsens between the examinations, the concept does not apply.

Several criteria can be considered by the employer in evaluating whether the PLHCPs involved are seeing the same condition:

- a. Whether examination of the patient is in person (i.e. review of documents only cannot be substituted for a physical examination);
- b. Whether the examinations are contemporaneous;
- c. Whether the employee was subject to additional exposures between the examinations; and
- d. Whether medical treatment, restricted work activity, or days away from work occurred between the examinations.

If all of the above criteria are met, the employer may rely on the most authoritative medical opinion. If these criteria are not met, this may indicate the PLHCPs are not seeing the same condition. OSHA would consider the medical treatment and days away from work directed by the second physician as necessary unless the employer can document that the first opinion was based on the exact same condition and is most authoritative. (See FAQ [7-10a](#) and OSHA's February 25, 2011 LOI, [Clarification of the terms most authoritative and pre-existing conditions as used for recording purposes.](#))

7. **Days Away from Work.** Section 1904.7(b)(3) addresses how to record a work-related injury or illness when it involves one or more days away from work. If a physician or licensed health care professional recommends days away, an employer should encourage the employee to

follow that recommendation and count those days as days away from work. Section 1904.7(b)(3)(ii) provides that the employer must count those days as days away whether the injured or ill employee follows the recommendation or not.

8. **Restricted Work.** Under section 1904.7(b)(4)(i), an employee's work is considered restricted when, as a result of a work-related injury or illness: (1) the employer keeps the employee from performing one or more of the routine functions of their job (job functions that the employee regularly performs at least once per week), or from working the full workday that they would otherwise have been scheduled to work; or (2) a physician or other licensed health care professional recommends that the employee not perform one or more of the routine functions of their job, or not work the full workday that they would otherwise have been scheduled to work. Note that several OSHA LOIs provide clarifications on whether to record days as restricted work or days away from work.

- a. **Employer-restricted work** - If an employee has a work-related injury or illness, and that employee's work is restricted by the employer to prevent exacerbation of, or to allow recuperation from, that injury or illness, the case is recordable as a restricted work case. For example, an employee who sustains burns to their arm while working in a radiological area is treated on-site with over-the-counter medication and a bandage and released with no restrictions in performing their routine job functions. The company, in accordance with its policy, restricts the employee from working in the radiological area for two days until his burn scabs over. This would be considered a restricted work case. (See OSHA's February 12, 2015 LOI regarding section 1904.7, [Clarification regarding the applicability of the recording criteria involving restricted work.](#))

However, a case is not recordable under section 1904.7(b)(4) as a restricted work case if the employee experiences minor injury or illness – for example musculoskeletal discomfort, and a health care professional determines that the employee is fully able to perform all their routine job functions, but the employer assigns a work restriction to that employee for the purpose of preventing a more serious condition from developing. (See FAQ [7-19.](#))

- b. **Recommendations from physician or other licensed health care professional** - If a physician or licensed health care professional determines an employee may work with restricted work duties, the employer must evaluate the restriction in light of the routine

job functions to determine whether the restriction keeps the employee from performing any job functions. (See section 1904.7(b)(4)(iv).)

As with days away, if a physician or other licensed healthcare professional recommends a job restriction that affects an employee's routine job functions, but the employee does all of their routine work, the employer must still record the injury or illness as a restricted work case. (See section 1904.7(b)(4)(viii).)

Conversely, if the physician or licensed health care professional recommends restricted work but the employee decides to stay home, such time should be counted as restricted work rather than days away if the employer determines the employee was able to come to work and perform restricted work. However, if an employer has no restricted work available to an employee placed on restricted work and sends the employee home, that time should be counted as days away from work. (See, for example, OSHA's August 3, 2006 LOI regarding section 1904.7, [Recording an injury when physician recommends restrictions but no restricted work is available.](#))

9. **Transfer to Another Job.** Under section 1904.7(b)(4)(ix), if an employer assigns an injured or ill employee to a job other than their regular job for part of the day, the case involves transfer to another job. This does not include the day on which the injury or illness occurred. For example, if an employer assigns, or a physician or other licensed health care professional recommends an employer to assign, an injured or ill worker to their routine job duties for part of the day and to another job for the rest of the day, the injury or illness involves a job transfer. Under section 1904.7(b)(4)(x), both job transfer and restricted work cases are recorded in the same box on the OSHA 300 Log.
10. **Medical Treatment Beyond First Aid.** Work-related injuries and illnesses that result in medical treatment beyond first aid meet the general recording criteria. Section 1904.7(b)(5)(i) provides that medical treatment means the management and care of a patient to combat disease or disorder. However, for purposes of Part 1904, medical treatment does not include: (1) visits to a physician or licensed healthcare professional solely for observation or counseling; (2) the conduct of diagnostic procedures, such as x-rays and blood tests, including the administration of prescription medications used solely for diagnostic

purposes (e.g., eye drops to dilate pupils); and (3) “first aid,” as defined in section 1904.7(b)(5)(ii).

For purposes of Part 1904, section 1904.7(b)(5)(ii) provides that first aid includes:

- a. Using a nonprescription medication at nonprescription strength; (For medications available in both prescription and non-prescription form, a recommendation by a physician or other licensed health care professional to use a non-prescription medication at prescription strength is considered medical treatment for recordkeeping purposes. See section 1904.7(b)(5)(iv) and see FAQ [7-8](#).)
- b. Administering tetanus immunizations; (Other immunizations, such as hepatitis B vaccine or rabies vaccine, are considered medical treatment.)
- c. Cleaning, flushing or soaking wounds on the surface of the skin;
- d. Using wound coverings, such as bandages, Band-Aids®, gauze pads, etc.; or using butterfly bandages or Steri-Strips® (other wound closing devices, such as sutures, staples, etc. are considered medical treatment);
- e. Using hot or cold therapy;
- f. Using any non-rigid means of support, such as elastic bandages, wraps, non-rigid back belts, etc.; (Devices with rigid stays or other systems designed to immobilize parts of the body are considered medical treatment for recordkeeping purposes.)
- g. Using temporary immobilization devices while transporting an accident victim (e.g., splints, slings, neck collars, back boards, etc.);
- h. Drilling of a fingernail or toenail to relieve pressure, or draining fluid from a blister;
- i. Using eye patches;
- j. Removing foreign bodies from the eye using only irrigation or a cotton swab;
- k. Removing splinters or foreign material from areas other than the eye by irrigation, tweezers, cotton swabs or other simple means;
- l. Using finger guards;
- m. Using massages (physical therapy or chiropractic treatment are considered medical treatment for recordkeeping purposes); and
- n. Drinking fluids for relief of heat stress. (However, note that intravenous administration of fluids to treat heat stress is medical treatment. See Recordkeeping FAQ [7-6](#).)

Any treatment not included on this list is considered medical treatment beyond first aid for OSHA recordkeeping purposes, and thus would be recordable.

Note: OSHA considers the above treatments to be first aid regardless of the professional status of the person providing the treatment. Consequently, even when the treatments are provided by a physician or other licensed healthcare professional, they are still considered first aid for purposes of Part 1904.

The application of a first aid treatment detailed in section 1904.7(b)(5)(ii) to treat a work-related injury, or illness is considered first aid for purposes of OSHA recordkeeping, regardless of the number of times the treatment is applied. In other words, if an injured or ill employee is given first aid treatment, such as hot or cold therapy, massage, or some other treatment on the first aid list, that treatment should not be considered medical treatment beyond first aid for OSHA recordkeeping purposes, even when such treatment is provided over a long period of time or involves multiple applications. For example, if a work-related injury or illness is treated only with multiple doses of a nonprescription medication (e.g., ibuprofen or acetaminophen) at nonprescription strength, and the employee continues to work a full day and perform all routine job functions, the injury or illness does not result in medical treatment and is not recordable.

However, repeated application of treatments included on the Part 1904 first aid list might be an indication that further necessary medical care is not being provided to an injured or ill employee. In such a case, the employer might be using repeated applications of first aid as a way to avoid the use of “medical treatment beyond first aid.” (See additional guidance in OSHA’s May 2, 2024 Memorandum, [Enforcement Guidance Under OSHA's Recordkeeping Regulation When First Aid, Active Release Techniques \(ART\), and Exercise/Stretching Are Used to Treat Musculoskeletal Injuries and Illnesses.](#))

11. **Diagnosis of a Significant Injury or Illness.** Section 1904.7 requires employers to record a significant work-related injury or illness diagnosed by a physician or other licensed health care professional, even if it does not result in death, days away from work, restricted work or job transfer, medical treatment beyond first aid, or loss of consciousness.

Section 1904.7(b)(7) specifies which significant, diagnosed work-related injuries and illnesses OSHA require recording. They are: work-related cancer, chronic irreversible disease such as silicosis or byssinosis,

punctured eardrum, or fractured or cracked bone. OSHA has also clarified that this includes fractured teeth. (See OSHA's 2001 *Federal Register* Notice, Vol. 66, page 5995). This list is exhaustive; no other conditions are considered a significant injury or illness for recordkeeping purposes under this provision.

12. **Needlestick and Sharps Injuries.** Section 1904.8 addresses the recording of work-related injuries and illnesses involving punctures, cuts and lacerations caused by needles or other sharp objects contaminated or reasonably anticipated to be contaminated with blood or other potentially infectious materials that may lead to bloodborne disease such as AIDs, hepatitis B or hepatitis C. The recordkeeping regulation uses the terms "contaminated," "other potentially infectious materials," and "occupational exposure" as these terms are defined in OSHA's Bloodborne Pathogens standard at 29 CFR 1910.1030.

Additional compliance guidance is outlined in 29 CFR 1904.8 of the recordkeeping regulation and section X. of OSHA Instruction [CPL 02-02-069](#), Enforcement Procedures for the Occupational Exposure to Bloodborne Pathogens.

- a. **Using the OSHA Forms 300 and 301 to record needlestick and sharp injuries** - Section 1904.8(a) requires employers to record on the OSHA 300 Log all work-related needlestick and sharps injuries involving objects contaminated (or reasonably anticipated to be contaminated) with another person's blood or other potentially infectious material. Employers may use the OSHA Forms 300 and 301 to meet the sharps injury log requirement in OSHA's Bloodborne Pathogens standard at 29 CFR 1910.1030(h)(5), if the employer enters the type and brand of the device causing the sharps injury on the OSHA 300 Log and maintains the records in a way that segregates sharps injuries from other types of work-related injuries and illnesses or allows sharps injuries to be easily separated.

When recording cases under section 1904.8, paragraph (a) prohibits the employer from entering the affected employee's name on the OSHA 300 Log. Instead, the employer must enter "privacy concern case" in the space reserved for the employee's name. The employer then keeps a separate, confidential list of privacy concern cases with the case number from the OSHA 300 Log and the corresponding employee name. (See the requirements for privacy concern cases in section 1904.29(b)(6) through(b)(9).)

- b. **Cuts caused by non-contaminated sharp instruments** - Although the regulation requires the recording of all workplace cut and puncture injuries resulting from an event involving contaminated sharps, it does not require the recording of all cuts and punctures. For example, a cut made by a knife or other sharp instrument that was not contaminated (or reasonably anticipated to be contaminated) by blood or other potentially infectious material would not generally be recordable, and a laceration made by a dirty tin can or greasy tool would also generally not be recordable, providing that the injury did not result from a contaminated sharp and did not meet one of the general recording criteria. Section 1904.8(b)(2) includes provisions indicating which cuts and punctures must be recorded because they involve contaminated sharps, and which must be recorded only if they meet the general recording criteria in section 1904.7.
 - c. **Updating OSHA Form 300 where injury leads to bloodborne illness** - Section 1904.8(b)(3) contains requirements for updating the OSHA 300 Log when a worker experiences a wound caused by a contaminated needle or sharp and is later diagnosed as having a bloodborne illness, such as AIDS, hepatitis B or hepatitis C. The regulation requires the employer to update the classification of such a privacy concern case on the OSHA 300 Log if the outcome of the case changes, i.e., if it subsequently results in death, days away from work, restricted work, or job transfer. The employer must also update the case description on the Log to indicate the name of the bloodborne illness and to change the classification of the case from an injury (i.e., the needlestick) to an illness (i.e., the illness that resulted from the needlestick).
 - d. **Contact with blood or infectious materials not involving needlesticks or sharps** - Section 1904.8(b)(4) addresses the recording of cases involving workplace contact with blood or other potentially infectious materials that do not involve needlesticks or sharps, such as splashes to the eye, mucous membranes, or non-intact skin. The recordkeeping regulation does not require employers to record these incidents unless they result in the diagnosis of a bloodborne illness or meet one or more of the general recording criteria in section 1904.7.
13. **Cases Involving Medical Removal.** Section 1904.9 requires an employer to record a case on the OSHA 300 Log if an employee is medically removed from the workplace under the medical surveillance

requirements of an OSHA standard. The employer must enter each medical removal case on the OSHA 300 Log as a case involving either days away from work (if the employee does not work during the medical removal) or as restricted work activity (if the employee continues to work, but in an area where exposures are not present). If the medical removal is the result of a chemical exposure, the employer must enter the case on the Form 300 Log by checking the “poisoning” column.

The following standards have medical surveillance requirements which include medical removal requirements:

- a. Benzene. General industry Standard (section 1910.1028(i)); Shipyard standard (§1915.1028); and Construction Standard (section 1926.1128)
- b. Cadmium. General Industry Standard (section 1910.1027(l)); Shipyard Standard (section 1915.1027); and Construction Standard (section 1926.1127)
- c. Formaldehyde. General Industry Standard (section 1910.1048(l)); Shipyard Standard (section 1915.1048); and Construction Standard (section 1926.1148)
- d. Lead. General Industry Standard (section 1910.1025); Shipyard Standard (section 1915.1025); and Construction Standard (section 1926.62)
- e. Methylenedianiline. General Industry Standard (section 1910.1050(m)); Shipyard Standard (section 1915.1050); and Construction Standard (section 1926.60(n))
- f. Methylene Chloride. General Industry Standard (section 1910.1052(j)); Shipyard standard (section 1915.1052); Construction standard (section 1926.1152)
- g. Vinyl Chloride. General industry Standard (section 1910.1017(k)); Shipyard Standard (section 1915.1517); and Construction Standard (section 1926.1117)
- h. Beryllium. General Industry Standard (section 1910.1024(l)); Construction Standard (section 1926.1124(l), and Maritime Standard (section 1915.1024(l)).

Note: In some cases, employers voluntarily rotate employees from one job to another to reduce exposure to hazardous substances. Section 1904.9(b)(3) provides that an employer is not required to record a case of medical removal on the OSHA 300 Log when the case involves voluntary medical removal before the medical removal levels required by an OSHA standard are reached.

14. **Cases Involving Occupational Hearing Loss.** Section 1904.10(a) requires employers to record work-related hearing loss cases when: (1) the

employee's audiogram reveals a work-related Standard Threshold Shift (STS) in hearing in one or both ears; and (2) the employee's total hearing level is 25 decibels (dB) or more above audiometric zero (averaged at 2000, 3000, and 4000 hertz (Hz)) in the same ear(s) as the STS. An STS is defined in OSHA's Occupational Noise Exposure Standard at 29 CFR 1910.95(g)(i) as "a change in hearing threshold, relative to the baseline audiogram for that employee, of an average of 10 dB or more at 2000, 3000, and 4000 (Hz)" in one or both ears.

Section 1904.10(b)(5) provides that there are no special rules for determining the work-relatedness of hearing loss. Employers must use the rules in section 1904.5 to determine if the hearing loss is work-related. A case is work-related if an event or exposure in the work environment either caused or contributed to the hearing loss, or significantly aggravated a pre-existing hearing loss. It is not necessary for the work environment to be the sole cause, or even the predominant cause, of the hearing loss in order for the case to be work-related. Any contribution from work to the hearing loss makes a hearing loss case work-related.

When evaluating the work-relatedness of a given hearing loss case, employers should take several factors into consideration, including:

- a. The employee's prior occupational and non-occupational noise exposure.
- b. Calibration records and the audiometric testing environment.
- c. The employee's records, including medical records.
- d. The employee's workplace activities, age correction, and personal medical conditions.
- e. If an employee is working in a high-noise environment, whether the employee is consistently and properly using hearing protection devices (e.g., is the employee using the protection, does it fit properly, and is it appropriate for the type of noise to which the employee is being exposed). (See OSHA's April 29, 2016 LOI, [Recording criteria for cases involving occupational hearing loss when employees use hearing protection.](#))

Note: Many employers may be conducting audiometric testing pursuant to OSHA's Noise Standard at 29 CFR 1910.95. However, employers not covered by section 1910.95 (e.g., construction, agriculture, oil and gas servicing and drilling), but who voluntarily conduct audiometric testing of employees, are also required to record a work-related hearing loss case if the hearing loss meets the two-step criteria in Section 1904.10(a).

15. **Work-Related Tuberculosis Cases.** Section 1904.11 requires employers to record a case on the OSHA 300 Log when an employee has been occupationally exposed to anyone with a known case of active tuberculosis (TB), and that employee subsequently develops a TB infection, as evidenced by a positive skin test or diagnosis by a physician or other licensed healthcare professional. For these cases, employers must check the “respiratory condition” column.

Employers may line-out or erase recorded TB cases from the OSHA 300 Log if the case is not occupational. Section 1904.11(b)(2) lists the following circumstances to assist employers to rule out cases where occupational exposure is not the cause of the infection in the employee:

- a. The employee is living in a household with a person who has been diagnosed with active TB;
- b. The Public Health Department identified the employee as a contact of an individual with a case of active TB unrelated to the workplace; or
- c. A medical investigation shows that the employee’s infection was caused by exposure to TB away from work or proves that the case was not related to the workplace TB exposure.

Additional information regarding the requirement to record work-related TB cases is provided in section 1904.11 and OSHA Instruction [CPL 02-02-078](#), Enforcement Procedures and Scheduling for Occupational Exposure to Tuberculosis.

16. **Musculoskeletal Disorders (MSDs).** Work-related injuries and illnesses involving muscles, nerves, tendons, ligaments, joints, cartilage and spinal discs are recordable under the same requirements applicable to any other type of injury or illness. These injuries are also referred to as musculoskeletal disorders, or MSDs. There are no special rules for recording these cases; if the case is work-related, a new case, and involves medical treatment, days away, job transfer or restricted work under the criteria in section 1904 described above in this section, it is recordable. As is the case with other injuries or illnesses, to avoid a potential repetitive stress injury, an employer may choose to reassign an employee with a work-related MSD condition before it becomes severe enough to result in medical treatment beyond first aid, days away, restricted work, or job transfer. (See also, section 1904.7(b)(4), the discussion above addressing restricted work under paragraph IX.E.8. of this instruction, and additional guidance in OSHA’s May 2, 2024 Memorandum, [Enforcement Guidance Under OSHA's Recordkeeping Regulation When First Aid, Active Release Techniques \(ART\), and](#)

[Exercise/Stretching Are Used to Treat Musculoskeletal Injuries and Illnesses.](#))

F. Deadline for Entering a Case.

Section 1904.29(b)(3) states that the employer must enter each case on the OSHA 300 Log and OSHA 301 Form within 7 calendar days of receiving information that a recordable injury or illness has occurred. In most cases, employers know immediately or within a short time that a recordable case has occurred. In a few cases, however, it may be several days before the employer is informed that an employee's injury or illness meets one or more of the recording criteria. If the employer receives more information after entering a case, section 1904.33(b)(1) allows an employer to revise an entry on the OSHA 300 Log simply by lining it out or amending it if further information justifying the revision becomes available.

Note: There is an exception to the 7-day recording period if an employee experiences a recordable hearing loss, and the employer elects to retest the employee's hearing within 30 days. In that case, under section 1904.10(b)(4), the employer can wait for the results of the retest before recording the case on the OSHA 300 Log and Form 301 Incident Report.

G. Employee Privacy.

Section 1904.29(b)(6) and (b)(7) include requirements for employers to protect the privacy of injured or ill employees when recording cases in certain types of cases. In these cases, the employer may not enter the injured or ill employee's name on the OSHA 300 Log. Instead, the employer must enter "privacy case," and keep a separate, confidential list containing the identifying information for those individuals. If the work-related injury involves any of the following, it must be recorded as a privacy case:

1. An injury or illness to an intimate body part or the reproductive system;
2. An injury or illness resulting from a sexual assault;
3. A mental illness;
4. HIV infection, Hepatitis, or Tuberculosis;
5. Needlestick and sharps injuries that are contaminated with another person's blood or other potentially infectious material as defined by 29 CFR 1910.1030 (see additional discussion in paragraph IX.E.12. of this instruction); or
6. Other illnesses, if the employee independently and voluntarily requests that their name not be entered on the OSHA 300 Log.

This is a complete list.

Note: The employer may be required to provide the OSHA Forms 300 and 301 records to government representatives, employees, former employees or

regulation allows employers to petition OSHA if they want to maintain records in a manner that is different from the approach required in 29 CFR 1904, rather than following the procedures in 29 CFR 1905. Section 1904.38 requires that employers show that their alternative recordkeeping system: (1) collects the same information as required by the Part 1904 regulation; (2) meets the purposes of the OSH Act; and (3) does not interfere with the administration of the OSH Act. (See section 1904.38 for specific requirements and variance application and approval procedures.)

P. Reporting Severe Injuries and Illnesses to OSHA.

Section 1904.39(a)(1) requires employers to report to OSHA within 8 hours each work-related fatality, and Section 1904.39(a)(2) requires employers to report to OSHA within 24 hours each work-related inpatient hospitalization, amputation, and loss of an eye. The reporting requirements in section 1904.39 apply to all employers covered by the OSH Act, including those that are partially exempt from keeping OSHA injury and illness records because of size or industry classification.

1. **Methods For Reporting.** Section 1904.39(a)(3) requires employers to report the work-related fatality, in-patient hospitalization, amputation, or loss of an eye using one of the following methods:
 - a. By telephone or in person to the OSHA Area Office that is nearest to the site of the incident;
 - b. By telephone to the OSHA toll-free central telephone number, 1-800-321-OSHA (1-800-321-6742); or
 - c. By electronic submission using the Serious Event Reporting Online Form reporting application located on OSHA's public website, entitled [Report a Fatality or Severe Injury](#).

Note: If the Area office is closed, the employer must still report the fatality, in-patient hospitalization, amputation, or loss of an eye in a timely manner, by using either the OSHA telephone number or the reporting application. (See 1904.39(b)(1).)

Note: If an employer reports to OSHA an in-patient hospitalization, amputation, or loss of an eye, and the employee subsequently dies within 30 days of the incident, the employer is not required to make a second report regarding the fatality. (See OSHA's January 8, 2021, LOI, [Reporting two related reportable events](#).)

2. **Information Required in Reporting.** Section 1904.39(b)(2) requires employers to give OSHA the following information for each fatality, in-patient hospitalization, amputation, or loss of an eye:
 - a. The establishment name;

- b. The location of the work-related incident;
- c. The time of the work-related incident;
- d. The type of reportable event (i.e., fatality, in-patient hospitalization, amputation, or loss of an eye);
- e. The number of employees who suffered a fatality, in-patient hospitalization, amputation, or loss of an eye;
- f. The names of employees who suffered a fatality, in-patient hospitalization, amputation, or loss of an eye;
- g. The employer's contact person and phone number; and
- h. A brief description of the work-related incident.

3. **Circumstances in which Reporting is Not Required.** Under section 1904.39, employers do not have to report an event to OSHA if any of the following circumstances apply:

- a. It is not work-related.
- b. It occurred more than 30 days after the work-related incident.
- c. It resulted from a motor vehicle accident on a public street or highway, except in a construction work zone. (However, employers must report events occurring in construction work zones.)
- d. It occurred on a commercial or public transportation system (e.g. airplane, subway, bus, ferry, streetcar, light rail, or train).

Note: Events that are not required to be reported to OSHA may still be recordable if the employer is required to keep Part 1904 Injury and Illness Records and the event meets the recording criteria in the regulation. (See sections 1904.4 through 1904.11, and prior discussion of these provisions in this instruction for recordability requirements.)

- 4. **Heart Attacks.** Section 1904.39(b)(5) requires employers to report work-related fatalities and in-patient hospitalizations caused by a heart attack. However, the local OSHA Area Office Director will decide whether to investigate the event, depending on the circumstances of the heart attack.
- 5. **Delays in Reporting.** Under section 1904.39(b)(7), if an employer is not immediately aware of a reportable incident, a report to OSHA must be made within 8 hours after a fatality is reported to the employer or any of its agents, and 24 hours after an in-patient hospitalization, amputation, or loss of an eye is reported to the employer or any of its agents. Under section 1904.39(b)(8), if an employer is not immediately aware that a reportable incident is the result of a work-related incident, the employer must similarly report to OSHA within 8 hours after the employer or any of its agents learns this in the event of a fatality, and within 24 hours after

the employer or any of its agents learn about an in-patient hospitalization, amputation, or loss of an eye.

Q. Providing Records to Government Representatives.

Section 1904.40(a) states that employers must provide a complete copy of any records required by Part 1904 to an authorized government representative when requested. Copies of the requested records must be provided within 4 business hours. Records under Part 1904 include the OSHA Form 300 (Log), the OSHA Form 300A (Summary), the confidential listing of privacy concern cases along with the names of the injured or ill privacy case workers, and the OSHA Form 301 (Incident Report). Section 1904.40(b)(1) specifies that authorized government representatives include (1) a representative of the Secretary of Labor conducting an inspection or investigation under the OSH Act, (2) a representative of the Secretary of Health and Human Services (including the National Institute for Occupational Safety and Health (NIOSH)) conducting an investigation under Section 20(b) of the OSH Act, or (3) a representative of a State agency responsible for administering an OSHA-approved State Plan.

Note: The government's right to ask for such records is limited by the authority of that Agency. For example, a representative of a State Plan can only ask for the records when visiting an establishment within that State.

Note: Under section 1904.40(b)(2), if the employer maintains the records at an establishment in a different time zone, the employer may use the business hours at the location where the records are being retained when calculating the deadline.

R. Electronic Submission of Injury and Illness Data to OSHA.

Section 1904.41 requires certain employers to electronically submit to OSHA, on an annual basis, injury and illness data that they are already required to keep under Part 1904. These data are submitted through OSHA's Injury Tracking Application (ITA).

1. **Establishments Required to Electronically Submit Data.** The electronic submission requirement is based on establishment size and industry classification.
 - a. **OSHA Form 300A** - Section 1904.41(a)(1)(i) requires establishments with 20-249 employees at any time during the previous calendar year, that are classified in an industry listed in [Appendix A to Subpart E of Part 1904](#), to electronically submit to OSHA information from their OSHA Form 300A Summary of Work-Related Injuries and Illnesses. Section 1904.41(a)(1)(ii) requires establishments with 250 or more employees at any time during

Appendix B

Compliance Officer Checklist

This checklist is a resource that provides recommended steps for CSHOs to take in conducting a recordkeeping evaluation during an OSHA inspection.

PRE-INSPECTION PREPARATION:

Review Inspection History.

- ☐ Before initiating an inspection, review the employer's inspection history and take other preparatory steps as contemplated in [Chapter 3 of OSHA's Field Operations Manual \(FOM\)](#), CPL 02-00-164.
- ☐ If possible, check for the establishment NAICS code and the number of employees in the entire company to determine recordkeeping coverage.

NOTE: Companies with 10 or fewer employees at all times during the previous calendar year or the establishment's industry is classified in one of the 2007 NAICS codes listed in Appendix A to Subpart B are exempt from maintaining the OSHA injury and illness records.

Check ITA data for the establishment.

- ☐ Obtain any Injury Tracking Application (ITA) information available. Each Area Office has access to the ITA database. ITA information is available on OSHA's [ITA data webpage](#).

Obtaining Administrative Subpoenas and Medical Access Orders.

- ☐ In some circumstances, it may be necessary to issue an administrative subpoena to obtain evidence related to an OSHA inspection or investigation. For more information on issuing an administrative subpoena, review [Chapter 15 of the FOM](#).
- ☐ In some circumstances, a medical access order will be needed for OSHA personnel to examine and copy personally identifiable employee medical information. For more information on medical access orders, review [OSHA Instruction CPL 02-02-072](#), Rules of Agency Practice and Procedure Concerning OSHA Access to Employee Medical Records, published August 22, 2007.

ON-SITE RECORDS REVIEW:

Ask the employer for the following Information.

- ☐ Ask for the OSHA 300 logs, 300A summary, and 301 incident reports for current and three prior calendar years.

- ☐ Ask for a roster of current employees.
- ☐ Check if the establishment has an on-site medical facility, nursing, health, or first aid facility. If so, it may be appropriate to request the company first aid log.
- ☐ Ask where the nearest emergency department is located where employees may be treated for workplace injuries and illnesses.
- ☐ Additional records to review may include employers' written policies related to recordkeeping and reporting, any medical records maintained by the employer, workers compensation records, insurance records, first report of injury, company first-aid logs, and/or nurse/physician logs.

Verify NAICS Code and Entire Company Size.

- ☐ Verify the accuracy of the establishment's NAICS code and the number of employees in the entire company and enter this information in OIS. (See above note about partial recordkeeping exemption.)

Enter OSHA 300 data into OIS to calculate DART Rate.

- ☐ CSHOs will not normally need to calculate the Days Away, Restricted, or Transferred (DART) rate since it is automatically calculated when OSHA 300 data are entered into the OIS. However, if it is necessary to manually calculate the DART rate, consult [Chapter 3 of the FOM](#) for further information.

Conduct Records Review.

- ☐ Review and compare information on the employers' OSHA 300, 300A, and 301 forms and other information received to identify any trends, potential hazards, types of operations and work-related injuries or illnesses, and to determine whether the employer properly recorded and reported information to OSHA in compliance with 29 CFR 1904.

In addition, compare the establishment's ITA data with the OSHA 300A, 300 and 301 forms (or equivalent forms) for the previous calendar year.

Some factors that may indicate a potential for inaccurate recording, or that may warrant closer review of the establishment's records, are included in the Reviewing Establishment Injury and Illness Records section of the Part 1904 Recordkeeping Policies and Procedures Directive.

- ☐ Where the records review indicates potential trends in types of incidents or hazards, in such situations it may be appropriate for the CSHO to conduct further evaluation of the

potential hazards during the inspection, including through the walkaround, employee interviews, and medical records review where appropriate.

NOTE: In some cases, the need for an administrative subpoena, or to obtain medical information through a Medical Access Order, may become apparent in the course of the records review or the inspection. See above section on Obtaining Administrative Subpoenas and Medical Access Orders.

NOTE: OSHA can expand the scope of the inspection when there is reasonable belief, based on specific evidence (e.g., injuries or illnesses recorded in both OSHA forms 300 and 301, employee statements, or “plain view” observations), that violative conditions may be found in other areas of the workplace. During a partial inspection, ordinarily, injury and illness data from the OSHA 300 logs *alone* will not be sufficient to support a broader inspection. However, OSHA 300 data in conjunction with other specific evidence—including incident report information from OSHA 301 forms, employee statements, or plain view observations—can be used to support an expanded inspection when the particular injuries or illnesses found in the OSHA 300 logs can be tied to a specific violative condition in the workplace.

EMPLOYEE INTERVIEWS:

- ☐ Conduct employee interviews in a manner consistent with [Chapter 3 of the FOM](#). In conducting a recordkeeping evaluation, the following may be helpful:
- ☐ Ask questions related to the employer’s recordkeeping policies and practices in interviews with the Management Representative and employees.
- ☐ Interview employees with reported and/or recorded injuries and illnesses, and inquire about injuries and illnesses in the context of all employee interviews.
- ☐ Where applicable, interview the Designated Recordkeeper, first-aid providers, and on-site healthcare professionals.
- ☐ In interviews, ask what procedures have been established at the location for employees to report work-related incidents.
- ☐ Ask if the company uses temporary or contract employees. If so, does the company supervise them on a daily basis? If yes, are their injuries and illnesses recorded on the OSHA 300 log?
- ☐ Ask questions to determine whether there may be company policies that have the effect of discouraging recording and reporting of injuries and illnesses. See OSHA’s November 10, 2016 Memorandum, [Interim Enforcement Procedures for New Recordkeeping Requirements under 29 CFR 1904.35](#) and October 11, 2018 Memorandum, [Clarification](#)